PEDIATRIC HISTORY

1-SOURCE OF HISTORY

2-PERSONAL DATA: -Name -Age -Sex -Nationality -Date of admission (OPD – ER)
-Any known illness (Duration + Medication)

3-MAIN COMPLAINT + DURATION

4-HISTORY OF PRESENTING ILLNESS (HPI): (check the note)

-Complaint analysis -Associated symptoms
-Previous episodes -Review of related system
-Risk factors
-Special questions related to the differential diagnosis / Important negatives
-History of any chronic disease (check the note)

5-HOSPITAL COURSE:

-What happened since admission: Improving - Worsening
-Investigations done + Findings -Medications given
-New diagnosis -New complaints
-Plan (or reason for hospital stay)

6-SYSTEMIC REVIEW: (Symptoms in light blue are for older children)

-General: Weak crying – Wight loss – Poor feeding – Irritability – Fever – Pallor - Lethargy
-CVS: Dyspnoea (Breathless & sweaty on feeding – Slow to feed)
Blue episodes – Dizzy spells – Fainting - Palpitation – Chest pain
-Respiratory: Sore throat – Ear ache – Cough (dry or Productive) – Heamoptysis – Wheeze
Dyspnoea - Frequent chest infections – Stridor (Noisy breathing)
-GIT: Appetite – Weight loss – Nausea - Vomiting – Heamatemesis - Abdominal pain
Dysphagia - Jaundice - Diarrhea - Constipation – Blood/Mucus in stool – Pruritis ani
Travel aboard – Animals contact – change in stool color – Recent fast food intake
-Genitourinary: Enuresis (dry during daytime?) – Nocturia – Dysuria (Crying during micturation)
Heamaturia – Abnormal genitalia – Urine color change - Age of menarche (female)
-CNS: Dizziness – Nervousness – Fits (Convulsion), Faints or Funny turns
Drowsiness – Weakness, Clumsiness or Frequent falls – Abnormal movements
Vision, Hearing, Taste & Smell problems - Incontinence – Tingling
Numbness or Unpleasant sensation - Headache
-Rheumatology: Limping – Bone pain – Joint (pain – Swelling – Redness) – Muscle pain
-Hematology: Epistaxis – Easy bruising – Circumcision bleeding – Petichae or Echemosis
Gum bleeding
-Dermatology: Rash – Itching – Pigmentation – Lump – Hair or Nail changes
-Endocrine: Polyuria – Polydepsia – Polyphagia

7-PAST HISTORY: -Medical -Surgical
-Hospital Admissions -Blood transfusion

8-ALLERGIES

9-PERINATAL HISTORY: -Antenatal: Pregnancy (Complications – Drugs – Radiation) – Gestation
-Natal: Mode of delivery – Birth weight
-Post natal: Diseases (e.g. Jaundice) – Admission to special care
Ventilation – Discharged (With mom / Stayed. .Why?)

10-NUTRITIONAL HISTORY:

-Bottle or breast fed . . . for how long?
-Timing of introduction of solids or cereals
-Current Dietary intake (Any dietary restriction?)
-Bottle fed (Type – Preparation – Volume – Duration – Frequency – Total daily intake)

11-IMMUNIZATION HISTORY: -Immunization card (Up to date?) -Last vaccine taken
-If not received . . . Why? -Extra Immunization
-Allergies or side effects from any vaccine

12-DEVELOPMENTAL HISTORY: (Compared to siblings) (Check the note)
-Gross motor -Fine motor & Visual -Social -Speech & Hearing

13-MEDICATION HISTORY: -Name -Duration -Dose -Side effects

14-SOCIAL HISTORY:
-Family social status -Financial support
-Home environment -Travel history -Pets at home
-Patient social status:
-Schooling (Which school – What sort – Level - Grades)
-Does the patient miss school? How frequent?
-How does he get on with other children?
-Effect of his chronic illness on his school performance

15-FAMILY HISTORY:
-Similar illness in family -Living where?
-Siblings (Number – Age – Sex – order – Healthy or . . .)
-Parents (Age – Education – Occupation – Income – Diseases – Consanguinity – Smoking)

16-SUMMARY: -Name -Age -Sex -Main complaint + Duration -Known illnesses
-Associated symptoms -Important findings (depend on the case)

Note: (HPI)
-If the problem of the patient was present since birth, then it is better to start your HPI from birth (start with the perinatal history) until the day of admission.
(How to ask about developmental history?)
-Start asking about the skills that should be performed at his age, if acquired then no need to ask the rest of the skills, however, if not acquired then ask about the skills that should be performed at younger age till you find out the latest skills the patient acquired.
-Ask if the patient lost previously acquired skills.
-If the patient goes to school, don't ask about these skills. However, you should ask about school performance, getting along with children, problems in walking, handling things or daily activity.
(History of any chronic disease)
-Since when?
-Diagnosed in which hospital?
-What were the presenting symptoms?
-What investigations were done to confirm the diagnosis?
-Medications being used / Surgeries were done.
-Improving / Worsening with the medications.
-Medications compliance? Home monitoring of the disease? By what? What are the usual readings?
-Following up in which hospital? Who is the treating doctor?
-Chronic or persisting symptoms.
-Exacerbations of the disease (Acute attacks – Acute complications)? Precipitating factors?
-Hospital admissions / ICU admissions.
-Complications? Follow up in other clinics to treat the complications.

DONE BY:
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